

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	An Open-label Phase 1 Study to Investigate SGN-CEACAM5C in Adults With Advanced Solid Tumors
Short Title:	A Study of SGN-CEACAM5C in Adults With Advanced Solid Tumors
Protocol Number:	331839925337539851
NCT Number:	NCT06131840
Sponsor Name:	Pfizer
Investigational Product:	bevacizumab
Phase:	PHASE1
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Number of participants with adverse events (AEs): An AE is any untoward medical occurrence in a clinical study participant, temporally associated with the use of study intervention, whether or not considered related to the study intervention 2. Number of participants with laboratory abnormalities 3. Number of dose modifications due to AEs
Secondary Obj.:	1. Pharmacokinetic (PK) parameter - Area under the concentration-time curve (AUC) 2. PK parameter - Maximum concentration (Cmax) 3. PK parameter - Time to maximum concentration (Tmax)

2.2 Background & Rationale

This clinical trial is studying advanced solid tumors. Solid tumors are cancers that start in a part of your body like your lungs or liver instead of your blood. Once tumors have grown bigger in one place but haven't spread, they're called locally advanced. If your cancer has spread to other parts of your body, it's called metastatic. When a cancer has gotten so big it can't easily be removed or has spread to other parts of the body, it is called unresectable. These types of cancer are harder to treat.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE1, Status: RECRUITING
Target N:	494

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

1. Tumor type:

* Participants in Part A (dose escalation) and Part B (dose optimization) must have histologically- or

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cytologically-confirmed metastatic or unresectable solid tumor malignancy. Must have relapsed, refractory, or progressive disease, and should have no appropriate standard therapy available.

- * Participants in Part A must have one of the following tumor types: colorectal cancer (CRC); gastric carcinoma (GC) or gastroesophageal junction adenocarcinoma (GEJ); non-small cell lung cancer (NSCLC); or pancreatic ductal adenocarcinoma (PDAC).

- * The tumor types to be enrolled in Part B will be identified by the sponsor from among those specified in Part A.

- * Participants in Part C (dose expansion) must have one of the following histologically- or cytologically-confirmed metastatic or unresectable solid tumor malignancies.

- * CRC and must have received prior treatment (in 1 or more lines of therapy) containing fluoropyrimidine, oxaliplatin, and irinotecan.

- * PDAC and must have received 1 prior line of therapy and received no more than 3 prior lines of therapy in the advanced or metastatic setting.

- * GC or GEJ and must have received prior platinum and fluoropyrimidine-based chemotherapy.

- * NSCLC and must have received platinum-based therapy. If eligible and consistent with local standard of care must have received a PD-1/PD-L1 inhibitor. In addition, participants with tumor genomic mutations/alterations for which approved targeted therapies are available per local standard of care, must have received such therapies.

- * Small cell lung cancer (SCLC) and must have received platinum-based therapy for extensive-stage disease and no more than 3 prior lines of therapy. If eligible and consistent with local standard of care must have received a PD 1/PD-L1 inhibitor.

- * Participants in Part D (bevacizumab combination therapy dose escalation) must have histologically confirmed unresectable or metastatic adenocarcinoma of the colon or rectum. Must have demonstrated progressive disease or intolerance to their last treatment regimen. Prior treatment regimens must have included fluoropyrimidine, irinotecan, oxaliplatin, an anti-VEGF monoclonal antibody and/or an anti-EGFR monoclonal antibody for RAS wildtype participants.

- * Participants in Part E (bevacizumab combination therapy dose expansion) must have histologically confirmed unresectable or metastatic adenocarcinoma of the colon or rectum. Must have received a maximum of 2 prior chemotherapy regimens for the treatment of advanced CRC and had demonstrated progressive disease or intolerance to their last regimen. Prior treatment regimens must have included a fluoropyrimidine, irinotecan, oxaliplatin, an anti-VEGF monoclonal antibody and/or an anti-EGFR monoclonal antibody for RAS wildtype participants.

2. Participants enrolled in the following study parts should have a tumor site that is accessible for biopsy(ies) and agree to biopsy(ies) and/or submission of archival tissue:

- * Monotherapy dose optimization (Part B)

- * Monotherapy (Part C) and bevacizumab combination therapy (Part E) disease-specific expansion cohorts

3. An Eastern Cooperative Oncology Group (ECOG) Performance Status score of 0 or 1

4. Measurable disease per Response Evaluation in Solid Tumors (RECIST) v1.1 at baseline.

4.2 Exclusion Criteria

1. Previous exposure to CEACAM5-targeted therapy.

2. Prior treatment with an antibody-drug conjugate (ADC) with a camptothecin payload

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3. History of another malignancy within 3 years before the first dose of study intervention, or any evidence of residual disease from a previously diagnosed malignancy.
4. Active cerebral/meningeal disease related to the underlying malignancy. Participants with a history of cerebral/meningeal disease related to the underlying malignancy are allowed if prior central nervous system disease has been treated and the participant is clinically stable (defined as not having received steroid treatment for symptoms related to cerebral/meningeal disease for at least 2 weeks prior to enrollment and with no ongoing related AEs).

\> Criteria related to bevacizumab administration (participants in Parts D and E)

5. History of allergic reactions or hypersensitivity to bevacizumab or any of its excipients.
6. History of hypersensitivity to Chinese Hamster Ovary cell products or other recombinant human or humanized antibodies.
7. Serious non-healing wound, non-healing ulcer, or non-healing bone fracture.
8. Deep venous thromboembolic event within 4 weeks prior to enrollment
9. Known coagulopathy that increases risk of bleeding, bleeding diatheses.
10. History of any life-threatening VEGF-related adverse event

11. Study Identifiers & Governance

Primary ID: 331839925337539851
Registry Number: NCT06131840
Sponsor: Pfizer
Study Title: A Study of SGN-CEACAM5C in Adults With Advanced Solid Tumors

15. Sign-off & Approval

Principal Investigator: _____ Date: _____
Clinical Operations Lead: _____ Date: _____
Lead Statistician: _____ Date: _____